PATHO LAB
A LEGACY OF EXCELLENCE

PROCESSED AT :
HY PATHO LAB MUMBAI

Corporate Office : HY PATHO LAB CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name : **AJAY KEWAT**
Ref By : **DR. AHMED KHAN**
Reg No : **HL6301832209 / MH493**
Barcode : **HEN153950**
INV : AFB Stain (Z-N Stain) 1ST Sample FOR SPUTUM

Age : 30 Year | **Sex :** Male
Registered on : 31-08-2026 03:03 PM
Received on : 31-08-2026 04:28 PM
Reported on : 31-08-2026 11:20 PM

SAMPLE COLLECTED AT :
HEAL DIAGNOSTIC CENTRE

SAMPLE : SPUTUM

AFB STAIN (Z-N STAIN) 1ST SAMPLE FOR SPUTUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Type of Specimen	SPUTUM	.	. - .
AFB STAIN (Z-N STAIN) 1ST SAMPLE	NOT SEEN	.	. - .

Interpretation :

No acid fast bacilli / 100 Oil Immersion Fields	-	Negative
<9 AFB in 100 Oil Immersion Fields	-	+
10-99 AFB in 100 Oil Immersion Fields	-	++
1-9 AFB per Oil Immersion Field	-	+++
>10 AFB per Oil Immersion Field	-	++++

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

HY PATHO LAB
A LEGACY OF EXCELLENCE

PROCESSED AT :
HY PATHO LAB MUMBAI

Corporate Office : HY PATHO LAB
CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name : **AJAY KEWAT**
Ref By : **DR. AHMED KHAN**
Reg No : **HL6001832514 / MH493**
Barcode : **HEN153953**
INV : AFB Stain (Z-N Stain) 2ND Sample FOR SPUTUM

Age : 30 Year | **Sex :** Male
Registered on : 31-08-2026 06:34 PM
Received on : 31-08-2026 07:02 PM
Reported on : 31-08-2026 11:21 PM

SAMPLE COLLECTED AT :
HEAL DIAGNOSTIC CENTRE

SAMPLE : SPUTUM

AFB STAIN (Z-N STAIN) 2ND SAMPLE FOR SPUTUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Type of Specimen	SPUTUM	.	. - .
AFB STAIN (Z-N STAIN) 2ND SAMPLE	NOT SEEN	.	. - .

Interpretation :

No acid fast bacilli / 100 Oil Immersion Fields	-	Negative
<9 AFB in 100 Oil Immersion Fields	-	+
10-99 AFB in 100 Oil Immersion Fields	-	++
1-9 AFB per Oil Immersion Field	-	+++
>10 AFB per Oil Immersion Field	-	++++

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

PATHO LAB
A LEGACY OF EXCELLENCE

PROCESSED AT :
HY PATHO LAB MUMBAI

Corporate Office : HY PATHO LAB CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name : **AKHTAR HUSAIN**
Ref By : **DR. AHMED KHAN**
Reg No : **HL1501832513 / MH493**
Barcode : **HEN153954**
INV : AFB Stain (Z-N Stain) 3RD Sample FOR SPUTUM

Age : 35 Year | **Sex :** Male
Registered on : 31-08-2026 06:33 PM
Received on : 31-08-2026 07:02 PM
Reported on : 31-08-2026 11:19 PM

SAMPLE COLLECTED AT :
HEAL DIAGNOSTIC CENTRE

SAMPLE : SPUTUM

AFB STAIN (Z-N STAIN) 3RD SAMPLE FOR SPUTUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Type of Specimen :	SPUTUM	.	. - .
AFB STAIN (Z-N STAIN) 3RD SAMPLE :	NOT SEEN	.	. - .

Interpretation :

No acid fast bacilli / 100 Oil Immersion Fields	-	Negative
<9 AFB in 100 Oil Immersion Fields	-	+
10-99 AFB in 100 Oil Immersion Fields	-	++
1-9 AFB per Oil Immersion Field	-	+++
>10 AFB per Oil Immersion Field	-	++++

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

HY PATHO LAB
A LEGACY OF EXCELLENCE

PROCESSED AT :
HY PATHO LAB MUMBAI

Corporate Office : HY PATHO LAB CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name : **JYOTI JAGTAP**
Ref By : **DR. BHARATI HOSPITAL**
Reg No : **HL4801821248 / MH026**
Barcode : **396706659**
INV : Albumin, Body Fluid

Age : 28 Year | **Sex :** Female
Registered on : 21-08-2026 10:52 AM
Received on : 21-08-2026 11:13 AM
Reported on : 22-08-2026 03:50 PM

SAMPLE COLLECTED AT :

SAMPLE : Body Fluid

ALBUMIN, BODY FLUID

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Type of Fluid	Ascitic fluid	.	. - .
Albumin	0.58	g/dL	. - Not Established

NOTE :

Reference range for Albumin is not established for body fluids. Physician to correlate clinically.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

HY PATHO LAB
A LEGACY OF EXCELLENCE

PROCESSED AT :
HY PATHO LAB MUMBAI

Corporate Office : HY PATHO LAB CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **DR SHABNAM SHAIKH**	**Age :** 48 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** NEW DIGNOSTIC
Ref By	: **DR. SHABNAM ABDUL**	**Registered on :** 29-08-2026 01:37 PM	
Reg No	: **HL3401830161 / RAJ43**	**Received on** : 29-08-2026 05:29 PM	
Barcode	: **HEJ018791**	**Reported on** : 29-08-2026 10:06 PM	
INV	: Anti Microsomal/Thryroperoxidase Antibody (AMA/Anti TPO) Titre		**SAMPLE :** Serum

ANTI MICROSOMAL/THRYROPEROXIDASE ANTIBODY (AMA/ANTI TPO) TITRE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
ANTI MICROSOMAL/THRYROPEROXIDASE ANTIBODY (AMA/ANTI TPO) TITRE Method: .	NEGATIVE	.	. - .
TITER Method: .	1.10	.	. - .

Clinical significance :
The most frequently detected Mitochondrial antibody which is of greatest clinical utility is M 2 antibody. Mitochondrial M2 antibodies have been detected in patients with Primary Biliary cirrhosis (96%) and occasionally in other Chronic liver diseases and Systemic sclerosis. These antibodies are also detected in asymptomatic patients who later go on to develop Primary Biliary cirrhosis

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

HY PATHO LAB
A LEGACY OF EXCELLENCE

PROCESSED AT :
HY PATHO LAB MUMBAI

Corporate Office : HY PATHO LAB CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **KONDU JADHAV**	**Age :** 65 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SMBS HOSPITAL , ZADPOLI**	**Registered on :** 26-08-2026 04:36 PM	SMBS HOSPITAL
Reg No	: **HL1401827488 / MH004/012/SAMBRE2025**	**Received on** : 27-08-2026 02:09 AM	
Barcode	: **HEE068019**	**Reported on** : 27-08-2026 11:06 AM	
INV	: Cholinesterase		**SAMPLE :** Serum

CHOLINESTERASE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Serum cholinesterase Method: Spectrophotometry	12.6	units/mL	8.00 - 18.00

Clinical significance :

The Serum Increased Level Is Seen In:

1. Nephrosis.
2. Diabetes mellitus.
3. Hyperlipidemia.

The Serum Decreased Level Is Seen In:

1. poisoning with organophosphate insecticide.
2. Liver diseases like hepatitis.
3. Cirrhosis with ascites or jaundice.
4. Metastatic carcinoma.
5. Obstructive jaundice.
6. Congestive heart failure.
7. Before you receive anesthesia with succinylcholine, which may be given before certain procedures or treatments, including electroconvulsive therapy (ECT).
8. In the condition with low serum albumins like malnutrition, anemia, infections, Dermatomyositis, Acute MI, and liver diseases.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

HY PATHO LAB
A LEGACY OF EXCELLENCE

PROCESSED AT :
HY PATHO LAB MUMBAI

Corporate Office : HY PATHO LAB CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **ANIKET BANGAR**	**Age :** 30 Year | **Sex :** Male		**SAMPLE COLLECTED AT :**	
Ref By	: **DR. SELF**	**Registered on**	: 27-08-2026 04:06 PM	BORA CLINICAL LABORATORY. LASALGAON. NASHIK	
Reg No	: **HL5701828495 / MH394**	**Received on**	: 28-08-2026 01:06 AM		
Barcode	: **HEJ192772**	**Reported on**	: 28-08-2026 08:05 AM		
INV	: MALE HORMONE PROFILE			**SAMPLE :** Serum	

DIHYDROTESTOSTERONE DHT FREE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
DIHYDROTESTOSTERONE DHT FREE Method: ELISA	2.7	pg/mL	1.00 - 6.20

Clinical significance :

Dihydrotestosterone measurement is used to diagnose 5α-reductase deficiency and for the evaluation of androgen utilization. Dihydrotestosterone is produced by the reduction of testosterone by 5α-reductase in the target organs. Circulating dihydrotestosterone is tightly bound to sex hormone-binding globulin. The free fraction represents the dihydrotestosterone available to act on tissues.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

HY PATHO LAB
A LEGACY OF EXCELLENCE

PROCESSED AT :
HY PATHO LAB MUMBAI

Corporate Office : HY PATHO LAB CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name : **BABY. SAFOORA**
Ref By : **DR. SELF**
Reg No : **HL5601825922 / MH525**
Barcode : **hen118477**
INV : EPSTEIN BARR VIRUS ANTIBODY TO VIRAL CAPSID ANTIGEN (VCA) IgM

Age : 6 Year | **Sex :** Female
Registered on : 25-08-2026 01:51 PM
Received on : 26-08-2026 12:13 PM
Reported on : 29-08-2026 11:04 AM

SAMPLE COLLECTED AT :
SERUM DIAGNOSTIC

SAMPLE : Serum

EPSTEIN BARR VIRUS ANTIBODY TO VIRAL CAPSID ANTIGEN (VCA) IGM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
EPSTEIN BARR VIRUS ANTIBODY TO VIRAL CAPSID ANTIGEN (VCA) IgM Method: EIA	6.95	U/mL	0.0 - 8.0

REFERENCE RANGE

RESULT IN U/mL	REMARKS
<8.00	Negative
8.00-12.00	Equivocal
>12.00	Positive

Interpretation

Epstein-Barr virus (EBV) is a virus that typically causes a mild to moderate illness. Blood tests for Epstein-Barr virus detect antibodies to EBV in the blood and help establish a diagnosis of EBV infection.

Epstein-Barr virus causes an infection that is very common. According to the Centers for Disease Control and Prevention (CDC), most people in the United States are infected by EBV at some point in their lives. The virus is very contagious and easily passed from person to person. It is present in the saliva of infected individuals and can be spread through close contact such as kissing and through sharing utensils or cups.

After initial exposure to EBV, there is a period of several weeks before associated symptoms may appear, called the incubation period. During the acute primary infection, the virus multiplies in number. This is followed by a decrease in viral numbers and resolution of symptoms, but the virus never completely goes away. Latent EBV remains in the person's body for the rest of that person's life and may reactivate but usually causes few problems unless the person's immune system is significantly weakened.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

HY PATHO LAB
A LEGACY OF EXCELLENCE

PROCESSED AT :
HY PATHO LAB MUMBAI

Corporate Office : HY PATHO LAB
CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **SAURABH KOTHARI**	**Age :** 40 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 02-09-2026 02:12 PM	SHREE LAB
Reg No	: **HL3301832634 / HYMH/KDV/H6569**	**Received on** : 02-09-2026 05:31 PM	
Barcode	: **HEE125360**	**Reported on** : 03-09-2026 06:33 AM	
INV	: FOLATE (FOLIC ACID), RBC		**SAMPLE :** EDTA Blood

FOLATE (FOLIC ACID), RBC

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Folate, Serum	7.9	ng/mL	. - >5.38
Folate, RBC	301	ng/mL	280 - 791

NOTE:

1. Drugs like Methotrexate & Leucovorin interfere with folate measurement
2. RBC folate concentration is indicative of tissue stores. It reflects the folate status over a period of 120 days.

Comments:

Folate plays an important role in the synthesis of purine & pyrimidines in the body and is important for the maturation of erythrocytes. It is widely available from plants and to a lesser extent organ meats, but more than half the folate content of food is lost during cooking. Folate deficiency is commonly prevalent in alcoholic liver disease, pregnancy and the elderly. It may result from poor intestinal absorption, nutrition deficiency, excessive demand as in pregnancy or in malignancy and in response to certain drugs like Methotrexate & anticonvulsants.

Decreased Levels:

Megaloblastic anemia, Infantile hyperthyroidism, Alcoholism, Malnutrition, Scurvy, Liver disease, B12 deficiency, dietary amino acid excess, adult Celiac disease, Tropical Sprue, Crohn's disease, Hemolytic anemias, Carcinomas, Myelofibrosis, vitamin B6 deficiency, pregnancy, Whipple's disease, extensive intestinal resection and severe exfoliative dermatitis

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

HY PATHO LAB
A LEGACY OF EXCELLENCE

PROCESSED AT :
HY PATHO LAB MUMBAI

Corporate Office : HY PATHO LAB CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **MS SAYED FABEHA BUSHRA**	**Age :** 20 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 12:02 PM	RESONANCE PATHOLOGY LAB
Reg No	: **HL1101829998 / MH734**	**Received on** : 29-08-2026 04:22 PM	
Barcode	: **HED094792**	**Reported on** : 31-08-2026 07:11 AM	
INV	: Free Androgen Index		**SAMPLE :** Serum

FREE ANDROGEN INDEX

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
FREE ANDROGEN INDEX	24.3	ng/dl	15.00 - 70.00

Method: ELISA

Clinical significance :

Free Androgen Index (FAI) is used as a surrogate marker for androgen status and is best interpreted along with other markers of androgen status, such as free testosterone SHBG, total testosterone and gonadotropins. FAI correlates well with free testosterone in women and in men, it is affected by changes in SHBG and fluctuations in total testosterone, based on circadian rhythm. In non-obese, non-hirsute oligomenorrheic women, an elevated AI during the early follicular phase is reported to be a sensitive and specific indicator of Polycystic Ovarian Disease (PCOD).

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

HY PATHO LAB
A LEGACY OF EXCELLENCE

PROCESSED AT :
HY PATHO LAB MUMBAI

Corporate Office : HY PATHO LAB CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name : **AMRITA BHARTI**
Ref By : **DR. TATA HOSPITAL**
Reg No : **HL5901826961 / MH081**
Barcode : **HEN102488**
INV : GASTRIN

Age : 40 Year | **Sex :** Female
Registered on : 26-08-2026 12:06 PM
Received on : 26-08-2026 04:43 PM
Reported on : 29-08-2026 08:28 AM

SAMPLE COLLECTED AT :
DNA LAB

SAMPLE : Serum

GASTRIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
GASTRIN, SERUM Method: CLIA	46.2	pg/ml	13.00 - 115.00

Clinical significance :

Gastrin is a primary gastrointestinal hormone which stimulates gastric acid secretion and exists in 3 principal forms namely G34 (big gastrin), G17 (little gastrin) & G14 (mini gastrin). Serum gastrin levels increase with increasing age specially in patients older than 60 years partly due to unrecognized gastric mucosal atrophy. Patients with Zollinger-Ellison syndrome show marked elevation upto 2000 times the normal gastrin level.
Increased Levels
ï¸· Gastrinoma / Zollinger-Ellison syndrome
ï¸· Pernicious anemia
ï¸· Pyloric obstruction with antrum distension
ï¸· Vagotomy without gastric resection
ï¸· Peptic ulcer disease
ï¸· Gastric carcinoma

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

hy PATHO LAB
A LEGACY OF EXCELLENCE

ISO 9001:2015

PROCESSED AT :
HY PATHO LAB MUMBAI

Corporate Office : HY PATHO LAB CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **OT SWAB**	**Age :** 1 Y | **Sex :** Male		**SAMPLE COLLECTED AT :**	
Ref By	: **DR. SHINDE HOSPITALBULDHANA**	**Registered on**	: 30-08-2026 10:36 AM		
Reg No	: **HL3701830926 / MH176**	**Received on**	: 30-08-2026 11:36 AM		
Barcode	: **R4177669**	**Reported on**	: 02-09-2026 10:52 AM		
INV	: OT Swabs C/S			**SAMPLE :** OT Swabs	

OT SWABS C/S

TEST DESCRIPTION	RESULT
Type of Specimen:	SWAB
Results Method: Disc Diffusion	No growth
Incubation period	obtained after 24 / 48 hrs of incubation at 37°c
Colony count	.

REMARKS :
Condition which can cause a false negative Cultures :

1. Patient is on Antibiotics.
2. Anaerobic bacteria.
3. Fastidious aerobic bacteria which are not able to grow on routine culture media.
4. Besides all these factors, at least in 25-40 % of cases there is on direct correlation between in vivo clinincal pictures and in-vitro culture result.

Interpretation :
(++++) or (+++) : maximum sensitivity
(++) : moderate sensitivity
(+) : minimum sensitivity
(R) : resistant

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. AMIT AGRAWAL (MD MICROBIOLOGY)
Consultant Microbiology

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

HY PATHO LAB
A LEGACY OF EXCELLENCE

PROCESSED AT :
HY PATHO LAB MUMBAI

Corporate Office : HY PATHO LAB CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **SANGEETA KOILKAR**	**Age :** 48 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** SHRIKANT MURCHAVDE
Ref By	: **DR. SELF**	**Registered on :** 29-08-2026 09:38 AM	
Reg No	: **HL3801829851 / MH031**	**Received on** : 29-08-2026 04:30 PM	
Barcode	: **HEN199498**	**Reported on** : 29-08-2026 09:55 PM	**SAMPLE :** Body Fluid
INV	: Uric Acid Synovial Fluid		

URIC ACID SYNOVIAL FLUID

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Uric Acid Synovial Fluid Method: PHOTOMETRY	5.0	mg/dL	0.0 - 8.0

Clinical Significance

Arthrocentesis with synovial fluid analysis should be attempted in all patients who have a joint effusion or signs suggestive of inflammation within the joint, without a known cause. The most important reason to perform a synovial fluid analysis may be to evaluate for septic arthritis in a patient presenting with an acutely swollen joint that is painful and warm even in patients with a known arthritic disease. (See "Septic arthritis in adults".)

Synovial fluid analysis is also particularly important for definitively diagnosing gout or pseudogout. In combination with a negative Gram stain and bacterial cultures, a diagnosis of gout or pseudogout may be established by confirming the presence of crystals of monosodium urate (MSU) or calcium pyrophosphate dihydrate (CPPD) in a patient presenting with unexplained inflammatory arthritis. (See "Clinical manifestations and diagnosis of gout" and "Clinical manifestations and diagnosis of calcium pyrophosphate crystal deposition (CPPD) disease".)

Synovial fluid analysis is also helpful in narrowing the differential diagnosis by distinguishing between categories of joint effusions (ie, inflammatory versus noninflammatory) (table 1), and to diagnose hemarthrosis (algorithm 1). (See 'Categories of joint effusions' below.)

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

HY PATHO LAB
A LEGACY OF EXCELLENCE

PROCESSED AT :
HY PATHO LAB MUMBAI

Corporate Office : HY PATHO LAB CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name : **NIYA MANJARE**
Ref By : **DR. SELF**
Reg No : **HL4601827012 / RAJ35**
Barcode : **HEN121310**
INV : Varicella-Zoster Virus (VZV) Chicken Pox-IgG

Age : 31 Year | **Sex :** Female
Registered on : 26-08-2026 12:22 PM
Received on : 26-08-2026 02:43 PM
Reported on : 26-08-2026 11:30 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

VARICELLA-ZOSTER VIRUS (VZV) CHICKEN POX-IGG

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
VARICELLA ZOSTER IgG	3.48	U/ml	0.00 - 8.00

Method: ELISA

Clinical significance :

Clinical significance :
Varicella-zoster virus (VZV), a herpes virus, causes 2 exanthematous (rash-associated) diseases, chickenpox and herpes zoster (shingles). Chickenpox is a highly contagious disease usually contracted during childhood and is characterized by a dermal vesiculopustular rash that develops in successive crops approximately 10 to 21 days following exposure.(1). Although primary infection results in immunity to subsequent exposure to chickenpox, the virus remains latent in the body, localized to the dorsal root or cranial nerve ganglia. Reactivation of latent infection manifests as herpes zoster. On reactivation, the virus migrates along neural pathways to the skin, producing a unilateral rash usually limited to a single dermatome. Reactivation occurs in older adults and in patients with impaired cellular immunity.(2)

Several populations are at risk of suffering unusually severe reactions to VZV infections. The infection in pregnant women may spread through the placenta to the fetus causing congenital disease in the infant. Immunocompromised patients in hospitals may contract severe nosocomial infections from others who have active VZV infections are at risk for developing severe VZV-related complications, which include cutaneous disseminated disease and visceral organ involvement.(2,3). Therefore, serologic screening of direct health care providers (physicians, allied health care personnel) and individuals in high-risk groups is necessary to avoid uncontrolled spread of infection.

While the clinical presentation of VZV infection is generally characteristic, serologic evaluation of patients with atypical and systemic infections is often required. For example, it is extremely important to serologically evaluate patients for the early detection of VZV infections in hospital settings. Nosocomial spread of VZV infection can be life-threatening to immunocompromised patients susceptible to infection

Please correlate with clinical conditions.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

HY PATHO LAB
A LEGACY OF EXCELLENCE

ISO 9001:2015

PROCESSED AT :
HY PATHO LAB MUMBAI

Corporate Office : HY PATHO LAB CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name : **RAJESH MAHALE.**
Ref By : **DR. SELF -**
Reg No : **HL9401822169 / MH103**
Barcode : **HEN125516**
INV : COMPLETE VITAMIN PROFILE

Age : 58 Year | **Sex :** Male
Registered on : 21-08-2026 11:33 PM
Received on : 22-08-2026 08:52 AM
Reported on : 24-08-2026 12:42 PM

SAMPLE COLLECTED AT :
ANNAPURNA LAB DHULE 9823099808

SAMPLE : Serum

VITAMIN K

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
VITAMIN K Method: Quantitative High Performance Liquid Chromatography	1.88	nmol/L	0.22 - 4.88

Clinical Information .

Vitamin K1 or phylloquinone is part of a group of similar fat soluble vitamins in which the 2-methyl-1,4-naphthoquinone ring is common. Phylloquinone is found in high amounts in leafy green vegetables and some fruits (avocado, kiwi). It is a required cofactor involved in the gamma-carboxylation of glutamate residues of several proteins. Most notably, the inactive forms of the coagulation factors prothrombin (factor II), factors VII, IX, and X and protein S and protein C are converted to their active forms by the transformation of glutamate residues to gamma-carboxyglutamic acid (Gla). Other proteins such as those involved in bone metabolism, cell growth, and apoptosis also undergo this Gla transformation. Measurement of vitamin K1 (phylloquinone) in fasting serum is a strong

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

PATHO LAB
A LEGACY OF EXCELLENCE

PROCESSED AT :
HY PATHO LAB MUMBAI

Corporate Office : HY PATHO LAB CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name : **MRS.SHOURYA JADHAV**
Ref By : **DR. KAPIL CHAVHAN SIR**
Reg No : **HL8401831114 / MH141**
Barcode : **HEN132307**
INV : Weil -Felix

Age : 02 Year | **Sex :** Male
Registered on : 30-08-2026 12:57 PM
Received on : 30-08-2026 01:07 PM
Reported on : 30-08-2026 03:15 PM

SAMPLE COLLECTED AT :
SEVARTH PATHOLOGY LAB AND ALLERGY TESTING CENTRE,WASHIM

SAMPLE : Serum

WEIL -FELIX

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Proteus Antigen OX 19 Method: Latex Agglutination	1:40	Titre	. - .
Proteus Antigen OX 2 Method: Latex Agglutination	1:40	Titre	. - .
Proteus Antigen OX K Method: Latex Agglutination	1:40	Titre	. - .

NOTE :

1. Titer of 1:160 is strongly suggestive of Rickettsial infection.

2. High levels of antibodies may be seen in patients with previous residual infection.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)